A ONE-PAGE PRE-VISIT BRIEF FOR INDIAN FOLLOW-UP CLINICS

The 60-second delta

The doctor has two minutes. **We give back the first sixty seconds.**

Not a records dashboard. The delta is the unit: what changed, what is unresolved, where sources disagree, and what the record cannot say. Every line opens its source. The doctor decides.

01 Problem

A follow-up visit begins with a reconstruction. The doctor opens old reports, prescriptions, a discharge summary, the lab portal and a WhatsApp thread while the patient repeats the story. About two minutes decide what gets seen. When that reconstruction is skipped, a change is missed, a test is repeated, or a referral quietly lapses.

02 Who is affected

Follow-up doctors in Indian diabetes, maternal and cancer clinics, in a government OPD and a private hospital alike. The record is there. The two minutes are not, and the patient carries the cost of the miss.

03 Solution

One page before the patient sits down. Four blocks: what changed, what is unresolved, where two sources conflict, and what we do not know. The doctor accepts, edits or flags an item. Accepted items become the visit note; unresolved items stay flagged for follow-up. The AI reads, matches and reconciles. It does not diagnose, recommend or score.

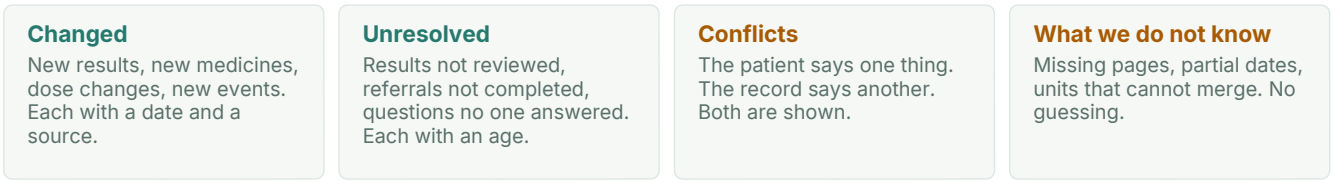


Figure 1. Native vector blocks, no image. Inside the product, every line beneath them opens its original page with the line highlighted.

04 The moment

The eGFR card opens its source in one click: 70 in April, 55 in July. The May 2025 eye review asked for a review in 12 months, and no later ophthalmology record exists. The product shows the interval and does not call it overdue. On the synthetic case, the read is 42 seconds: demo evidence, not a measured average.

05 Evidence

WHAT THE EVIDENCE SHOWS, AND ITS SOURCE	SOURCE
Follow-up visit: **2 minutes** in a nine-clinic government OPD study	IJCMH 2019
Trackable by phone and address: **11.2%** of 3,696 public-programme patients, Chennai audit	BMC Infectious Diseases 2018
Critical values notified late: **64.61%** of 1,187, 30 to 120 minutes after sample arrival	Indian J Clin Biochem 2015
Missed abnormal results: **6.8% to 62%** across the reviewed settings	J Gen Intern Med 2012
Critical alert requirements: **a register and a read-back**, retention of at least one year	NABL 112, NABH

06 KPI

Time to brief
Under 60 seconds per patient
TARGET

Delta completeness
Share of required delta items present, per case
TARGET

Unresolved closed
Items closed within 7 days of the visit
TARGET

Doctor confidence
Rated by the doctor after each brief
TARGET

All four are targets. None is measured yet. The pilot runs a manual baseline and the delta arm on the same cases, in balanced order.

The doctor sees one page, three decisions, and every source one click away. **The full ledger is there when they want it.**

07 How it works

Five steps from a bag of paper to a draft note. The doctor sees only the brief.

01	02	03	04	05
Ingest	**Extract**	**Bind**	**Reconcile**	**Approve**
PDFs, photos, WhatsApp forwards, lab portals	Facts, each with its evidence span	Every fact tied to a source line	Current against prior state	Doctor accepts, edits or flags

08 AI and safety

WHAT THE MODEL DOES

- Extraction, entity resolution, reconciliation, delta generation.
- Source binding on every generated line.

WHAT IT NEVER DOES

- No diagnosis, treatment advice or risk score.
- No autonomous action. No imputation of a missing value.

The clinic is the data fiduciary; the product is the processor. DPDP consent, notice, retention and audit. No medical device claim. No ABDM or WhatsApp integration claim. Documentation and retrieval aid, with human approval before any note is signed.

09 Market and business model

Beachhead	Diabetes follow-up in one Tier 2 cluster. One workflow, one user, one outcome.
Buyer	The clinic owner, or the hospital quality head who answers for critical results.
Price	₹1,500 per doctor per month; ₹3,000 per two-doctor clinic. The pilot price is fixed for eight weeks.
Expansion	Maternal, cancer, then government OPDs. Same delta, different clinical vocabulary.

10 Why now

- Extraction is finally good enough for Indian paper, scans and forwarded photographs.
- Records are digitised but unusable at the point of care.
- DPDP Rules make audit a requirement, not a feature. Cheap inference makes it affordable.

11 Why us

- A backend, worker, web client and Capacitor Android shell, already in the product.
- Versioned facts, evidence binding and an identity firewall in the data model.
- A clinical partner for the delta rubric, and a team that ships in days.

12 The ask

Three clinics, 100 follow-ups, eight weeks. A manual baseline and the delta arm on the same cases, in balanced order. A doctor partner to sign the delta rubric.

The pilot outputs the four KPIs, measured against a manual baseline.

13 Team

Clinical lead Clinical plausibility, the delta rubric, pilot sites.

Engineering lead Pipeline, evidence binding, pilot instrumentation.

Design and product The brief, the demo, the measurement.

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